INFORMATION TO KEEP WITH YOU

My complete treatment handbook

Weekly carboplatin + paclitaxel with radiotherapy

Understand your treatment. Know what to expect. Keep your plan to hand.

Displayed calendar
7 days
See the active treatment phase

Treatment stages
3
Explained in the treatment reference

Regimen source
NCCP 00309
Version 6a

Paclitaxel then carboplatin by drip once every 7 days for seven weeks, alongside separately scheduled radiotherapy. Test results or side effects may change treatment. Any treatment after this course needs a separate decision and prescription.

Treatment stages

01 **Seven weekly chemotherapy treatments**
Weekly ×7 with radiotherapy (RT)

02 **Radiation course**
Separate appointments

03 **After chemoradiation**
Review then next plan

Your team confirms the active stage and dates. The full treatment guide explains each step.

Inside your handbook

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My name

Hospital / oncology team

Use the urgent instructions and contact numbers in your treatment guide. Your team must replace example numbers before patient use. Your individual treatment plan takes precedence.

CHAPTER 01

Understanding your treatment

IN THIS CHAPTER

Why this treatment is offered, what its medicines do, and how the agreed plan fits your care.

When to get help

Emergency: call 112 / 999

Severe breathing difficulty, collapse, seizure or severe chest pain: call emergency services. Do not wait for an online answer.

Contact your oncology service immediately

Temperature 37.5°C or higher, 36°C or lower, shivering or suddenly feeling unwell — even with a normal temperature. Call oncology immediately for fever, shaking chills, sudden illness, new breathlessness, uncontrolled vomiting, inability to swallow fluids, much less urine or unusual bleeding. Severe breathing difficulty, chest pain, collapse or heavy bleeding needs emergency care. Tell staff immediately about infusion pain, swelling or allergy symptoms.

Follow the urgent contact instructions from your treating hospital. If you cannot reach the team, seek urgent medical assessment. Outside clinic hours, use your instructed out-of-hours pathway or the emergency department.

This page does not diagnose symptoms or decide whether it is safe to stay at home. You do not need to know which medicine caused a symptom before calling.

My chemotherapy medicines

Chemotherapy is given alongside radiotherapy to treat stage III non-small-cell lung cancer when cisplatin is unsuitable. Your team explains the treatment aim and choices.

TAXANE CHEMOTHERAPY

Paclitaxel

Taxane chemotherapy



Disrupts cell division

Paclitaxel stabilises structures needed for cell division. It can also affect nerves, blood cells and healthy tissues. Premedicines help reduce infusion reactions.

Treatment reaches cells → Disrupts cell division → Cancer growth may slow

What to watch for

Report numbness, tingling, muscle/joint pain or new breathing symptoms.

PLATINUM CHEMOTHERAPY

Carboplatin

Platinum chemotherapy



Damages deoxyribonucleic acid (the cell’s genetic material) (DNA)

Carboplatin forms links in DNA that interfere with cell copying. Its dose uses kidney filtration and target exposure(AUC), not a fixed number of milligrams.

Treatment reaches cells → Damages DNA → Cancer growth may slow

What to watch for

Report fever, unusual bleeding, vomiting or allergy symptoms.

The drawings are simplified explanations of drug action, not anatomical pictures or a prediction of response.

Why chemotherapy and radiotherapy are combined

Studies support combining systemic treatment with radiation in the relevant lung-cancer setting.

The evidence comes from specific treatment schedules and patient groups. Your team explains how it applies to your situation; study averages cannot predict an individual outcome.

CHAPTER 02

Your schedule, tests and appointments

IN THIS CHAPTER

Your treatment stages, checks and practical preparations, from before treatment through follow-up.

My treatment overall

Treatment stages

01 **Seven weekly chemotherapy treatments**
Weekly ×7 with radiotherapy (RT)

02 **Radiation course**
Separate appointments

03 **After chemoradiation**
Review then next plan

Your team confirms the active stage and dates. The full treatment guide explains each step.

This shows treatment timing, not guaranteed symptom-free days. Your team confirms the active phase and dates.

WEEKLY ×7 WITH RT

Seven weekly chemotherapy treatments

1 2 3 4 5 6 7

Both medicines are given by drip once every 7 days for seven weeks, alongside a separate radiotherapy course. The plan may change for side effects or test results. Further chemotherapy after radiotherapy is a separate decision.

SEPARATE APPOINTMENTS

Radiation course

Team-agreed

Radiation is given during the chemotherapy course. Its dose, number of fractions and precise timing are prescribed by radiation oncology. A chemotherapy delay does not automatically stop radiation.

REVIEW THEN NEXT PLAN

After chemoradiation

Review

Your team reviews recovery and response. NCCP describes optional separate two-cycle carboplatin/paclitaxel consolidation under00304; it is not automatically included and any next treatment needs a separate prescription.

The chemotherapy course is finite; radiotherapy timing and next treatment are confirmed separately.

BASELINE → TREATMENT → FOLLOW-UP

My tests and treatment checks

Required safety tests are distinct from team-agreed imaging and follow-up; no universal tumour-marker interval is specified.

The team arranges and reviews tests. This guide shows expected timing; it does not record completed appointments. NCCP 00309 v6a · official regimen

THE WHOLE TREATMENT JOURNEY

Weekly carboplatin + paclitaxel with radiotherapy: treatment and checks

Stage III non-small-cell lung cancer · concurrent chemoradiation

Both medicines are given by drip once every 7 days for seven weeks, alongside a separate radiotherapy course. The plan may change for side effects or test results. Further chemotherapy after radiotherapy is a separate decision.

Read from left to right. Columns show stages, not equal lengths of time. This is a general plan: no dates or personal information are needed. Your team confirms which steps apply and arranges appointments.

Treatment, tests and reviews across the course

Care / check	Before treatment	Treatment	Between visits	Next cycle / week	After treatment
Chemotherapy NCCP 00309	Confirm aim and prescription	One visit each week	No additional routine doses	Seven weekly treatments	Next treatment only if agreed
Radiotherapy NCCP 00309	Separate planning appointment	Alongside weekly treatment	Follow radiation appointments	RT team confirms ongoing plan	Review late symptoms
Tests NCCP 00309	Blood count / kidneys / liver	Review required tests	Extra tests for symptoms	Bloods every week	Follow abnormal results
Symptoms / response NCCP 00309	Baseline symptoms and contacts	Report new symptoms	Fever / breathlessness: urgent	Review tolerance	Individual scans / recovery

What each check is for

1 Prepare

Before first dose

Chemotherapy is given alongside radiotherapy to treat stage III non-small-cell lung cancer when cisplatin is unsuitable. Your team explains the treatment aim and choices.

Ask / act: Confirm prescription, radiation plan and contacts.

2 Monitor

Each week

Check fitness and tolerability

Ask / act: Report symptoms and confirm test results before dosing.

3

Review

After course / when clinically indicated

Assess response and recovery

Ask / **act:** Confirm the next plan and persistent symptoms.

At the next review, ask

- Which treatment phase am I in, and what happens next?
- Which test is expected next, who arranges it, and who explains the result?
- Who should I contact if an expected appointment has not arrived?

This map does not show whether a test has been ordered, booked or completed. Do not wait for a scheduled test if you develop new symptoms.

Practical treatment planning

Agree the arrangements with your oncology team.

STEP 1

Confirm your plan

Paclitaxel is infused first over one hour, then carboplatin over 30 minutes, plus checks and premedicines. The consultant may prescribe a lower paclitaxel dose. RT appointments are separate and are not counted as chemotherapy doses.

STEP 2

Keep the contact card

The team supplies the daytime and out-of-hours numbers. Keep the card with you and show it when seeking medical care.

STEP 3

Two linked treatment teams

Confirm which team to contact for radiation or chemotherapy changes. Report swallowing problems early and ask about transport, nutrition and fatigue support.

CHAPTER 03

Symptoms and supportive care

IN THIS CHAPTER

Recognising changes, knowing when to call, and using your prescribed supportive-care plan.

Day 1 → Day 7: a general pattern

This is a visual orientation, not a prediction of your personal cycle. It shows when some effects often emerge and which effects are **not** tied to a particular day.

Treatment or check	Timing in this view
Paclitaxel then carboplatin	 **Day 1** One weekly chemotherapy visit after blood tests and premedicines.
Symptoms and separate RT plan	 **Days 1–7** Continue the written radiation and support plan. Symptoms may occur on any day; this is not a radiation-fraction calendar.

Relative schedule, not personal dates: Both medicines are given by drip once every 7 days for seven weeks, alongside a separate radiotherapy course. The plan may change for side effects or test results. Further chemotherapy after radiotherapy is a separate decision.

No fixed symptom day: Sickness, fatigue, low counts, swallowing problems or breathing symptoms can occur between visits and after treatment. Do not wait for a calendar window to seek advice.

Radiation is a separate schedule: Chemotherapy Day 1/Day 2/Day 3 or weekly counts are not radiation fractions. The radiation oncology team supplies the exact RT timetable.

> **Why these bands stay broad:** Reference cycle without patient dates. RT visits and dose are separately prescribed; the symptom band is not a nadir prediction. Band length does not show likelihood or severity, and the chart cannot tell you whether it is safe to receive treatment. NCCP 00309 v6a · official regimen ↗

> **There are no guaranteed “safe days”.** Call oncology immediately for fever, shaking chills, sudden illness, new breathlessness, uncontrolled vomiting, inability to swallow fluids, much less urine or unusual bleeding. Severe breathing difficulty, chest pain, collapse or heavy bleeding needs emergency care. Tell staff immediately about infusion pain, swelling or allergy symptoms.

Between-treatment notes

Day 1: Weekly treatment day

Blood results and symptoms are reviewed. Paclitaxel comes first, followed by carboplatin. Tell staff immediately about flushing, breathing trouble, pain or swelling during infusion.

Day 2–3: Support at home

Use the anti-sickness medicines prescribed for these days. Radiation visits may continue on their own schedule. Call for fever, poor intake, vomiting or new breathing symptoms.

Day 4–7: Before the next weekly dose

Keep track of swallowing, fatigue, numbness and muscle/joint symptoms. Arrange the next weekly blood tests. Report problems before the next dose; do not wait if acutely unwell.

Support around the medicines

1 Weekly medicines before treatment

Paclitaxel needs premedicines to reduce allergic reactions. The first week and later weeks can differ. Your team also provides anti-sickness medicines for carboplatin; do not double up steroid doses.

2 Eating, swallowing and breathing

Chest radiation can make swallowing sore. Ask early for pain relief and nutrition support. New breathlessness, cough or inability to drink needs urgent advice, even after the course finishes.

3 Nerves and aches

Tell the team about numbness, tingling or muscle/joint pain before each weekly visit. Follow the agreed pain-relief plan and check before starting medicines or supplements.

Questions worth asking

Which drugs and routes are prescribed for me?

Confirm the exact drug names, routes and interval against your written prescription. The team must explain any change from this reference plan.

How do my radiation appointments fit around chemotherapy?

Radiation has its own calendar. Ask for the combined plan and whom to contact if a chemotherapy or radiation visit changes.

Which blood tests are required before the next dose?

Use the checks table as a prompt. Your team confirms which tests are needed, where they are done and who reviews the results before treatment.

Who do I call during the day and overnight?

Ask the team to replace the example numbers on your card with the correct daytime, overnight and backup contact routes before you leave.

When will recovery and response be reviewed?

Agree the follow-up appointments and any scans with your team. This regimen does not set a universal scan or tumour-marker timetable.

> **Try explaining the plan back:** Explain back your chemotherapy days, how radiation is scheduled separately, the tests before treatment and whom you will call if unwell.

Supportive prescription draft for review. This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Week 1 · antiemetic plus paclitaxel reaction prevention

Active medicines: Paclitaxel + Carboplatin

Emetogenic risk: Moderate

Medicine	Dose / route	When
Ondansetron	16 mg orally	Before weekly chemotherapy Review QT/constipation risk.
Dexamethasone antiemetic reference	8 mg orally Day 1 and Days 2–3	NCIS moderate-risk course Reconcile with paclitaxel steroid premedication; one approved schedule.
Metoclopramide	10 mg orally three times daily as needed	Subsequent days Check contraindications and duration.
Dexamethasone paclitaxel reference	8 mg intravenous (IV)	30 minutes before paclitaxel NCCP reaction-prevention dose; do not duplicate the antiemetic steroid.
Chlorphenamine	10 mg IV	30 minutes before paclitaxel
Famotidine	20 mg IV	30 minutes before paclitaxel Consultant/local-policy decision on omission.

Additional phase-specific support

- Reconcile paclitaxel premedication and antiemetic dexamethasone into ONE prescription; do not add duplicate steroids.
- NCCP allows consultant-led famotidine omission without prior hypersensitivity, while noting uncertain reaction risk; close monitoring and reaction facilities remain necessary.
- Muscle/joint pain: consider prescribed analgesia. Assess swallowing, nutrition and respiratory symptoms during and after radiotherapy (RT).
- No routine granulocyte colony-stimulating factor (white-cell growth support) (G-CSF), antibiotic, antiviral or Pneumocystis jirovecii pneumonia (PJP) prophylaxis is mandated in this source; assess individual risk/local policy.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

Day 8 onward · review prior reactions each week

Active medicines: Paclitaxel + Carboplatin

Emetogenic risk: Moderate

Medicine	Dose / route	When
Ondansetron	16 mg orally	Before weekly chemotherapy Review QT/constipation risk.
Dexamethasone antiemetic reference	8 mg orally Day 1 and Days 2–3	NCIS moderate-risk course Reconcile with paclitaxel steroid premedication; one approved schedule.
Metoclopramide	10 mg orally three times daily as needed	Subsequent days Check contraindications and duration.
Chlorphenamine	10 mg intravenous (IV)	30 minutes before paclitaxel
Famotidine	20 mg IV	30 minutes before paclitaxel May be omitted if no hypersensitivity under consultant guidance.
Paclitaxel-specific dexamethasone	None routinely in this source table from Day 8	May be added for increased risk or a prior reaction This does not cancel the separate carboplatin antiemetic steroid. Consultant may prescribe20 mg orally12 hours and6 hours before a carefully authorised rechallenge; severe paclitaxel reactions require separate specialist management and no routine rechallenge.

Additional phase-specific support

- Reconcile paclitaxel premedication and antiemetic dexamethasone into ONE prescription; do not add duplicate steroids.
- NCCP allows consultant-led famotidine omission without prior hypersensitivity, while noting uncertain reaction risk; close monitoring and reaction facilities remain necessary.
- Muscle/joint pain: consider prescribed analgesia. Assess swallowing, nutrition and respiratory symptoms during and after radiotherapy (RT).
- No routine granulocyte colony-stimulating factor (white-cell growth support) (G-CSF), antibiotic, antiviral or Pneumocystis jirovecii pneumonia (PJP) prophylaxis is mandated in this source; assess individual risk/local policy.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

What to watch for

You do not need to know which medicine caused a symptom before calling.

1 Nerves

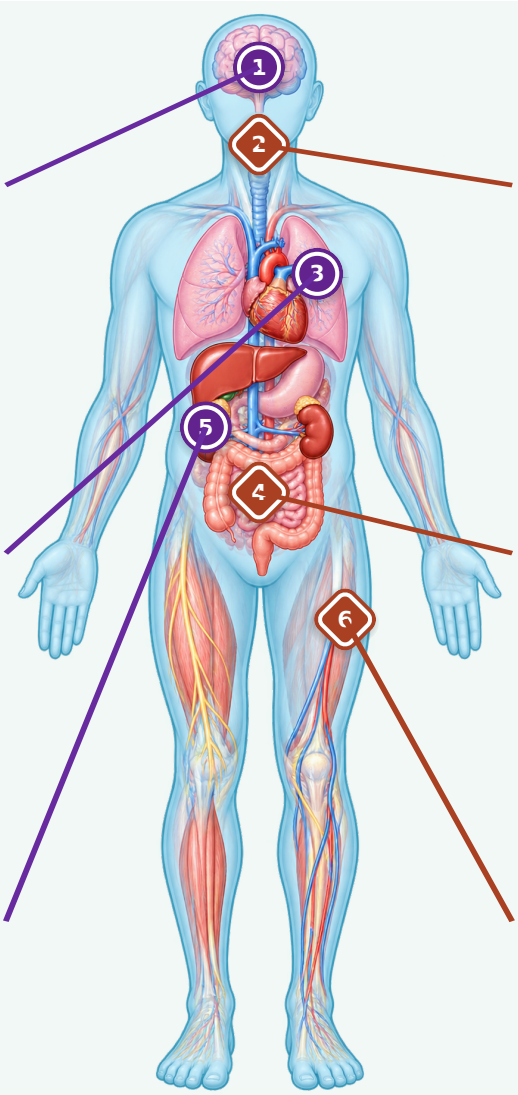
Paclitaxel

Report numbness, tingling, weakness or difficulty with everyday tasks before treatment.

2 Mouth / swallowing

Chemoradiation

Tell the team if swallowing becomes painful. Inability to drink needs urgent advice.



3 Lungs / allergy

Chemoradiation

New breathlessness or worsening cough needs urgent review, including after treatment ends.

4 Sickness / appetite

Chemotherapy

Use prescribed anti-sickness treatment; call for uncontrolled vomiting or dehydration.

5 Kidneys / dose checks

Carboplatin

Kidney tests help calculate the dose. Report reduced urine or dehydration.

6 Blood cells / muscles

Chemotherapy

Call for fever or bleeding. Report muscle/joint pain and any infusion-site swelling.

Selected effects, not every possible effect or a prediction. Blood-count, skin and line markers are schematic. Number and text carry meaning; colour is decorative.

Call oncology immediately for fever, shaking chills, sudden illness, new breathlessness, uncontrolled vomiting, inability to swallow fluids, much less urine or unusual bleeding. Severe breathing difficulty, chest pain, collapse or heavy bleeding needs emergency care. Tell staff immediately about infusion pain, swelling or allergy symptoms.

When to get help

Call 112 / 999 for severe breathing difficulty, severe chest pain, collapse or a seizure.

Call oncology immediately: temperature 37.5°C or above, 36°C or below, shivering or suddenly feeling unwell, even with a normal temperature. Follow your hospital's temperature instructions. If you cannot reach the team, seek urgent medical assessment. Do not wait for the next appointment.

Oncology daytime: 087 12345678 · **Out of hours:** 087 87654321

EXAMPLE ONLY — team must replace both numbers before patient use.

CHAPTER 04

Sources and further information

IN THIS CHAPTER

The references behind this information and addresses you can use to find further help.

Use this information alongside the plan agreed with your treatment team. Your team confirms your medicines, doses and appointments.

1. **National Cancer Control Programme (NCCP) 00309 v6a · official regimen**
   https://assets.hse.ie/media/documents/309_CARBOplatinAUC2_and_PACLitaxel50_weekly_with_radiotherapy (RT).pdf
2. **NCCP NCIS medical-oncology antiemetics v8**
   https://assets.hse.ie/media/documents/Supportive_Care_Antiemetics_for_inclusion_NCIS_Medical_Oncology.pdf
3. **Paclitaxel manufacturer product information**
   https://www.medicines.org.uk/emc/product/3891/smpc
4. **Choy et al.1998 · phase II weekly chemoradiation**
   https://pubmed.ncbi.nlm.nih.gov/9779707/